

NeuroFlo: Full Platform Overview

The Ultimate Value We NeuroFlo turns complex surgical documentation into instant, error-free revenue and operational data. We make surgeons faster, we get hospitals paid within 24 hours instead of weeks, and we turn dead data into live operational KPI insights and dashboards. We Aren't Automating Medical Coding. We Are Engineering Surgical Revenue From the Ground Up.

NeuroFlo is an end-to-end surgical revenue engineering platform designed specifically for surgical documentation, medical coding, and operational intelligence. It operates natively as a SMART on FHIR (Substitutable Medical Applications and Reusable Technologies on Fast Healthcare Interoperability Resources) application, integrating directly within Electronic Health Record systems like Epic and Cerner.

The Value Proposition

"Turning Surgical Documentation into Instant, Error-Free Revenue."

The platform transforms unstructured operative dictations into fully coded, audit-compliant charge items in real time. By linking AI dictation, custom hospital coding logic, and deep clinical analytics, the system eliminates charge lag, prevents revenue leakage, and gives health systems complete visibility into surgical performance.

The Problem (Why Now?) Hospital margins are razor-thin, yet millions are lost because surgeons hate writing operative notes. Complex surgeries (like a triple bypass) require meticulous documentation to bill correctly. Surgeons dictate from memory while fatigued, missing critical billing details. Medical coders then spend days deciphering these notes, causing massive billing backlogs (DNFB), outsourced contractor fees, and high claim denial rates.

The Solution (How We Fix It)

- **1. AI Operative Note Suite (The Capture):** Surgeons dictate or template their notes. Our AI analyzes the text in *real-time* against surgical sub-specialties. If a surgeon forgets to mention the specific valve stem used or the anesthesia time blocks, the AI flags it *before* sign-off, ensuring 100% of the procedure is accounted for.
- **2. Medical Coding Center (The Conversion):** The finalized note is instantly translated into CPT, ICD-10, and HCPCS codes, pushed through a custom hospital rules engine, and presented to coders in a clean UI. This cuts coding time from days to minutes and guarantees clean claims.
- **3. Operational KPI Engine (The Capitalization):** 80% of surgical data is trapped in text. We allow hospital admins to use a no-code, drag-and-drop board to type plain-English

queries (e.g., "Aortic valve duration per surgeon") and instantly generate real-time metrics pulled directly from the AI-analyzed op notes.

Neuroflo has 3 Core Pillars

These are the main tenants of Neuroflo that drive solutions to outpace any competition and operate in a different market.

1. AI Operative Note Suite (Capture)

- **What it does:** Provides surgeons with real-time dictation and templated note capture tailored for complex surgeries.
- **The Value:** It actively analyzes the note at the point of care to prevent revenue leakage. If critical clinical or billing details are missing, it prompts the surgeon to verify them *before* signing off, ensuring the procedure is completely accurate, compliant, and fully billable.

2. Medical Coding & RCM Hub (Convert)

- **What it does:** Acts as an intelligent companion tool that instantly translates signed operative notes into submission-ready medical codes, applying the hospital's specific compliance and grouping rules.
- **The Value:** By accelerating the workflow for human coders, it crushes the Discharged Not Final Billed (DNFB) lag from the 7-day industry average to under 24 hours. This unlocks millions of dollars in trapped cash and accelerates hospital liquidity.

3. Operational KPI Engine (Capitalize)

- **What it does:** Uses an AI feature extractor to mine the unstructured clinical text that is typically trapped inside operative notes.
 - **The Value:** Hospital leadership can use a no-code, drag-and-drop, and dictation interface to type plain-language queries and generate live dashboards. This provides immediate operational insights—like tracking morbidity rates or surgical times—without the need for expensive, manual chart audits.
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NeuroFlo Whole-Platform Value

- **NeuroFlo collapses the space between the operating table and the revenue cycle into a single, continuous intelligence loop.**
- **We make surgeons faster, get hospitals paid within 24 hours instead of weeks, and turn dead text into live operational intelligence.** This is the clearest whole-platform summary because it captures all three parts of NeuroFlo in one statement.
- **We aren't just automating medical coding; we are engineering surgical revenue and operational intelligence from the ground up.**
- **Uncaptured procedures, omitted implants, untracked OR time, and missing secondary codes permanently erase up to 3% of a hospital's surgical revenue. NeuroFlo enforces real-time completeness while the surgeon's knowledge of the case is fresh—capturing 100% of billable components before sign-off to recover millions in unbilled care."**
- **NeuroFlo links AI-assisted surgical documentation, custom hospital coding logic, and deep clinical analytics in one connected platform, eliminating charge lag, preventing revenue leakage, and giving health systems visibility into surgical performance.**
- **NeuroFlo structures the information at its inception, so the same intelligence captured during the operative note flows into medical coding and then into clinical, operational, and financial analytics.** This is the architectural advantage behind the continuous intelligence loop.
- **Built natively as a SMART on FHIR application, NeuroFlo integrates with Epic and Cerner Electronic Health Record systems, using only the necessary case information and returning completed documentation directly to the patient's chart.**
- **NeuroFlo is designed specifically around complex surgical workflows rather than generic clinical documentation, connecting surgical documentation, coding, revenue, and operational intelligence as one system.** The underlying platform is built around surgical specialties and sub-specialties rather than treating every clinical encounter the same way.
- **Government-contractor status provides an additional enterprise and institutional-readiness credential.** This comes from the additional information you provided, not the attached documents.
- **SOC 2 readiness is part of NeuroFlo's enterprise deployment path.** Because the SOC 2 process is not yet completed, we should present this as readiness/in-progress rather than claiming completed SOC 2 certification.

Core Value Drivers

- **The Documentation Gap (Zero Revenue Leakage):** In healthcare billing, the governing standard of the Centers for Medicare & Medicaid Services (CMS) and legal compliance is "if it is not documented, it was not done". Rushed or manual operative notes routinely omit 1% to 3% of billable components, such as secondary anatomical repairs, exact implant or stent specifications, and anesthesia time blocks. Downstream coders—whether human or Artificial Intelligence (AI)—cannot legally bill for unrecorded items, resulting in permanent top-line revenue loss.
- **Point-of-Care Completeness:** Uncaptured procedures, omitted implants, untracked OR time, and missing secondary codes permanently erase more than 3% of a hospital's surgical revenue. NeuroFlo enforces point-of-care documentation completeness while the case details are fresh, actively guarding operative note creation in real time. The system cross-references dictated text against legal frameworks, historical data, and sub-specialty requirements, prompting the surgeon to verify missing clinical variables before final sign-off. This guarantees 100% procedural, legal, and financial completeness at the exact point of care.
- **Accelerated Cash Flow & Liquidity:** Once a procedure is completed, charts enter Discharged Not Final Billed (DNFB) status—the delay during which cash is trapped waiting for manual coding. By ensuring notes are complete upon sign-off, NeuroFlo drops the DNFB charge lag from the industry average of 7 days down to under 24 hours. For a facility processing \$5,000,000 daily (\$1,825,000,000 annual revenue baseline), reducing lag by 6 days instantly frees up \$30,000,000 in immediate, debt-free working capital, saving \$1,500,000 annually in cost of capital at a 5% interest rate.
- **Reduced Administrative Overhead:** Achieving first-pass clean claim rates above 95% cuts claim denial rework costs (\$118 average cost per denied claim) and displaces reliance on third-party outsourced coding agencies, yielding over \$1,390,000 in direct annual administrative and labor savings.
- **Surgical Capacity Expansion:** Cutting documentation time from 30–45 minutes down to 5–10 minutes reclaims 30 minutes per procedure. Across a team of 20 Cardiovascular (CV) surgeons operating 250 days per year, this unlocks 2,500 additional surgical hours annually—generating \$10,000,000 in top-line revenue capacity and \$2,000,000 in net profit margin without hiring additional physicians.
- **EHR Integration & Human-in-the-Loop Governance:** Built natively as a Substitutable Medical Applications and Reusable Technologies on Fast Healthcare Interoperability Resources (SMART on FHIR) application, NeuroFlo launches directly inside Electronic Health Record (EHR) systems like Epic and Cerner without ingesting unnecessary Personally Identifiable Information (PII). The Medical Coding & Revenue Cycle

Management (RCM) Hub operates as an intelligent companion tool, keeping human coders in the loop to verify clean claims faster and satisfy compliance standards.

- **Investor Exit Strategy & Valuation Milestone:** Healthcare AI platforms trade at 10x to 15x Forward Annual Recurring Revenue (ARR). Securing 20 mid-to-large hospital systems at an average enterprise license of \$500,000 per year hits \$10,000,000 ARR, establishing a \$120,000,000 valuation at a conservative 12x multiple. Prime acquisition targets include EHR incumbents (Epic, Oracle Health/Cerner, MEDITECH), RCM leaders (Optum, R1 RCM, Waystar), and enterprise tech leaders (Microsoft Nuance, Google Health).